



[www.addictionpas.org](http://www.addictionpas.org)

August 11, 2026

Submitted electronically via [www.regulations.gov](http://www.regulations.gov)

Ann M. Sheehy, Principal Deputy Administrator  
Health Resources and Services Administration  
U.S. Department of Health and Human Services  
5600 Fishers Lane, Room 13N194  
Rockville, MD 20857

**RE: Request for Information, Training and Care Delivery Models for Safe Administration of Potential FDA-Approved Psychedelic Therapies in Ambulatory Clinical Settings (91 FR 43103; July 14, 2026)**

Dear Principal Deputy Administrator Sheehy:

On behalf of the Society of Addiction Medicine Physician Associates (SAMPAs), we appreciate the opportunity to respond to the Health Resources and Services Administration's (HRSA) Request for Information on training and care delivery models for the safe administration of potential FDA-approved psychedelic therapies in ambulatory clinical settings. SAMPAs is the national professional organization representing physician associates/physician assistants (PAs) who practice in addiction medicine. Our members deliver evidence-based care for substance use disorders and co-occurring mental health conditions across emergency departments, hospitals, primary care, and community settings, including Federally Qualified Health Centers (FQHCs) and other safety-net clinics.

We support HRSA's interest in identifying safe, effective, and equitable models for delivering future FDA-approved psychedelic therapies, particularly in ambulatory settings that serve medically underserved communities. Many patients with serious mental illness also have co-occurring substance use disorders; any future framework must therefore account for the clinical realities of addiction medicine and the existing PA workforce that already cares for these populations.

**Core Recommendation: Explicit Recognition of PAs as Licensed Medical Providers**

Any future guidance, technical assistance, or care-delivery standards should explicitly recognize **PAs as licensed medical providers** eligible to:

- Medically evaluate patients and independently determine their eligibility prior to administration
- Order, prescribe (where authorized by state law), and oversee administration of the therapy

- Provide or supervise medical monitoring during the dosing session
- Participate in post-administration medical follow-up
- Train and supervise other members of the interdisciplinary team

PAs are educated and licensed as medical providers with broad diagnostic, prescribing, and clinical management authority in all U.S. states. They already manage controlled substances, perform high-stakes risk assessments, and deliver complex behavioral health care in precisely the settings (FQHCs, rural health clinics, and Certified Community Behavioral Health Clinics) that this RFI prioritizes. Excluding or under-recognizing PAs would reduce access in the very communities HRSA seeks to reach. Naming PAs explicitly in any future guidance will avoid confusion about roles and responsibilities.

## **Workforce Training and Care Continuum**

### **Pre-administration screening, eligibility, and counseling**

A licensed medical provider (physician, PA, or nurse practitioner) should conduct or directly oversee the medical evaluation and risk stratification. Broad behavioral screening and more detailed diagnostic assessment may be appropriately shared across an interdisciplinary team, but medical clearance and identification of contraindications require medical training and accountability. Any of these providers should be able to complete the evaluation and make the eligibility determination; a PA's determination should not require a duplicative physician evaluation. Telehealth can support portions of the screening and counseling process when clinically appropriate and paired with in-person medical evaluation as needed. These hybrid models of virtual and in-person care have already proven successful in managing other complex health conditions.

### **Day-of drug administration**

A licensed medical provider should be available during the session. Continuous observation by appropriately trained personnel is essential given patients' acute medical and psychiatric vulnerability while under the effects of these agents. Telehealth or remote monitoring alone is not an adequate substitute for on-site medical capacity in most ambulatory settings. Training for this phase should emphasize medical emergency recognition and response, physiological monitoring, and management of acute psychological distress. The medical provider and trained staff must be licensed or otherwise authorized to provide care in the location where the session occurs.

### **Post-administration follow-up**

Follow-up should include both medical and psychotherapeutic components. Specialized training in evidence-based integration approaches is invaluable; licensed medical providers, including PAs, should remain involved in medical monitoring and coordination of care, particularly for patients with co-occurring substance use disorders.

### **General training models**

Competency-based models that combine didactic education, simulation, and supervised practicum are preferable to rigid degree-based restrictions. Certification or specialized training pathways should be open to PAs on the same terms as other medical providers. Any collaboration imposed by state regulation or other entities should be based on scope of practice

and demonstrated competency on a practice level rather than on professional title alone. Additionally, PAs should be allowed to participate as research investigators contributing to the growing body of evidence for these treatment modalities.

## **Ambulatory and Safety-Net Settings**

Basic requirements for safe administration in health centers and similar settings should include:

- Pre-session medical evaluation and eligibility determination by a licensed medical provider, including PAs.
- Adequate staffing to ensure continuous observation, along with the supplies necessary for a rapid medical response
- Secure storage, inventory control, and handling consistent with applicable controlled-substance regulations
- Monitoring of administered substances for purity and quality, consistent with the standards for other prescribed medications.
- Clear interdisciplinary communication protocols between medical and behavioral health team members

PAs are already integral to the workforce in these settings. Including them fully in any future model will expand, rather than constrain, capacity.

## **Technology-Enabled Approaches**

Artificial intelligence and other digital tools may usefully assist with screening support, risk flagging, training simulation, post-session symptom tracking, patient engagement, and data collection/analysis. They should not replace clinical judgment in determining eligibility or substitute for real-time monitoring of patients. Human oversight by licensed medical providers remains essential.

PAs have been an integral part of the healthcare landscape for nearly 60 years, supporting patients and communities in all settings. SAMPA stands ready to serve as a resource to HRSA on workforce training, interdisciplinary models of care, and the practical realities of delivering complex behavioral and addiction care in ambulatory and underserved settings, particularly as they relate to emerging psychedelic therapies. We appreciate your consideration of these comments and look forward to continued engagement as this important work proceeds.

Please direct any questions regarding these comments to Natasha Seliski, PA-C, MPAS and Arianna Campbell, DMSc, MPH, PA-C, CAQ-EM, DFAAPA, co-chairs of the SAMPA Public Health Policy Committee at [policy@addictionpas.org](mailto:policy@addictionpas.org).

Respectfully submitted,



Shani Wilson, PA-C  
President  
Society of Addiction Medicine Physician Associates (SAMPA)